



Urinary Catheter Management Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All clinical services	Medical, Nursing and Midwifery

2. Purpose

The purpose of this procedure is to outline the requirements for the management of patients with a urinary catheter.

Nurses and Midwives must have completed the Armada Health Service [Urinary Catheterisation Self-Directed Learning Package](#) and be deemed competent in the procedure before catheterising a patient unsupervised.

3. Background

- This procedure covers the following areas for adult patients excluding obstetrics:
 - Urethral Catheterisation
 - Urethral Catheterisation - Intermittent
 - Intermittent Self-Catheterisation
 - Suprapubic Catheterisation
- For information on obstetric and paediatric patients, refer respectively to [Women and Newborn Health Service](#) or [Child and Adolescent Health Service](#).

Catheterisation provides a mechanism for increased risk of urinary tract infection.

Urinary catheterisation procedures **must only** be undertaken by clinical staff suitably credentialed and competent for the specific intended procedure using aseptic technique.

- Indications for urinary catheter include:
 - Urinary retention management.
 - Monitoring urine output or renal function.
 - During or post-surgical procedure.
 - Bladder washout or irrigation.
 - Intravesical medication administration.
 - Urinary system investigations.
 - Quality of life issues: e.g., end of life, sexual functioning.

- Short term bladder management: e.g., post-urethral surgery and pelvic trauma.
 - Long term bladder management of the neurogenic bladder e.g., spinal cord injury.
- Patients with a urinary catheter should be closely observed for signs of infection which include:
 - Urethral pain or burning sensation.
 - Sensation of urgency to void.
 - Offensive smelling urine.
 - Discoloured, cloudy or macroscopic haematuria.
 - Elevated body temperature (fever).
 - Confusion.
 - Self-reported feeling generally unwell.
 - Discomfort in the lower back or loin area (flank or suprapubic discomfort).
 - Costovertebral angle tenderness (acute angle formed on either side of the back between twelfth rib and vertebral column)
 - Increased incontinence in the intermittent catheter patient.

4. Procedure

4.1 Urethral catheterisation – indwelling catheter

- Catheter selection, equipment required and insertion procedure are demonstrated thoroughly in the [Urinary Catheterisation Self-Directed Learning Package](#)

Urinary catheters **must not** be forced into the urethra. If a nurse is not able to successfully catheterise a patient on the first attempt, they should seek the assistance of a more senior colleague.

4.2 Urethral catheterisation – intermittent

- Not all patients will require an indwelling catheter. Intermittent catheterisation is where a catheter is introduced into the bladder to drain urine, and then removed once the bladder is empty.

The decision to insert an indwelling catheter or an intermittent catheter will depend on the result of a bladder assessment; refer to the [Armada Kalamunda Group \(AKG\) Bladder Management Procedure](#)

- Generally, the type of catheter used is different to an indwelling catheter.
- The procedure will be carried out as an aseptic procedure.
- Catheter selection, equipment required and insertion procedure are demonstrated thoroughly in the [Urinary Catheterisation Self-Directed Learning Package](#)

4.3 Intermittent self-catheterisation

Some patients will have learnt clean intermittent self-catheterisation (CISC) and manage successfully in their own home. Wherever possible, patients should be encouraged to continue to manage this independently.

Note – If a patient is performing CISC themselves in hospital it is undertaken as a clean procedure, not an aseptic procedure. However, due to the increased risk of acquiring an

infection in the hospital environment, the importance of good hand hygiene must be emphasised.

At home patients clean and re-use catheters.

While in hospital patients must discard their catheter after single use and must be supplied with enough catheters for their stay.

4.4 Suprapubic catheterisation

Initial insertion of a suprapubic catheter is only undertaken by a suitably qualified medical practitioner.

- Subsequent changes of suprapubic catheters can be undertaken by nursing staff trained and competent to perform the procedure.
- While management of a suprapubic catheter is essential the same as for a urethral catheter, the following points must also be considered:
- Continence Clinic SPC change
 - Clinical staff performing suprapubic catheter change must be aware of the urological history of the patient and understand the reason for initial insertion of the suprapubic catheter. If the patient has complex medical conditions, where possible the procedure should be completed at a time that coincides with a urologists presence onsite.
 - Prior to the first suprapubic catheter change after initial insertion, confirm the date of insertion as documented in the medical record. If a SPC requires changing prior to 4 weeks from the initial date of insertion, contact the referring Urologist for advice.
 - On initial presentation to the continence clinic the patient needs to be informed about the procedure and risks that may arise when attending their SPC replacement and written consent obtained and filed in the current medical record:
 - Perforation
 - Haematuria
 - Bowel injury
 - After initial written consent, verbal consent is to be obtained and documented in the medical record prior to each SPC change.
 - Patient preparation prior to SPC change - Spigot the SPC for no longer than 30 minutes prior to the change, to allow for accumulation of urine in the bladder. The accumulation of urine in the bladder will assist in identifying the correct placement of the new suprapubic catheter. **NB:** If patient is using a catheter valve, spigotting is not required.
 - If the patient experiences any problems/abnormalities during or post procedure e.g., bleeding at cystostomy site, haematuria, unresolving pain or discomfort, the patient is to be referred to the Emergency Department.

4.5 Cystostomy site management

- Renew the cystostomy dressing for first 24 hours only, then leave the site open.
- Wash the cystostomy in the shower daily or clean with 0.9% sodium chloride.

4.6 Discharge planning

- Liaise with the Continence Advisor to arrange first catheter replacement following initial insertion.
- Ensure there is a plan for domiciliary nursing services in place for routine catheter changes.
- Provide patient education which includes details on who to contact in the event of complications/emergency

5. Legislative context

The following documents are required to give effect to this procedure:

- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)
- EMHS [Infection Prevention and Management Policy EMHS: 34](#)
- EMHS [Consent Policy EMHS: 5](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Bladder Management – Continence Procedure AKG-PRO-0057-17](#)
- [Hand Hygiene Policy AKG-POL-0187-17](#)
- [Aseptic Technique Procedure -AKG-PRO-0045-14](#)

7. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Action 3.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard when preventing and managing healthcare-associated infections, and implementing the antimicrobial stewardship program to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 3 – Action 3.9 Aseptic technique
- The health service organisation has processes for aseptic technique that:
 - a. Identify the procedures where aseptic technique applies

- b. Assess the competence of the workforce in performing aseptic technique
- c. Provide training to address gaps in competency
- d. Monitor compliance with the organisation's policies on aseptic technique

9. Policy contact

Director Nursing and Midwifery

Enquiries relating to this procedure may be directed to:

Title: Nurse Coordinator
Department: Hospital Wide

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	23/05/2023	23/05/2026	Full review following SAC 1 recommendation

11. Authorisation

This procedure has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Helen Fullarton – Director Nursing and Midwifery
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